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Mental Space Psychology©

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SOMSP

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0.0 Introduction

Over the last twenty-five years the role of *space* in cognitive science became ever more prominent, as witnessed by series of publications and

congresses on *spatial cognition* (like the 6th International Conference on Spatial Cognition, Rome 2015). Beside a wealth of fresh perspectives on human and animal psychology, these publications and congresses show how little awareness the involved neuro-scientists, psychologists and linguists have of the existence of *spatial psychotherapies*. On the other hand, the therapists who work *spatial*, on their part, have only fragmented ideas about what science is discovering about *space*.

Spatial psychotherapies are certainly not an application of *spatial cognition*, since most therapies were developed in isolation and often before *spatial cognition* became an area of research. Which means that spatial psychotherapies are an independent development. In this study we explore what new insights this spatial clinical work provides about the 3 dimensions of the mind.

The name given to the combined study of *spatial cognition* and *spatial psychotherapies* is Mental Space Psychology (MSP). In brief, MSP investigates how humans orient themselves in the physical world (spatial cognition), how they find their way in their inner psychological universe (by spatial indexing, spatial storage, retrieval and imagery) and how space is used in the field of clinical psychology (in what we will call here *spatial psychotherapies*).

The drive to investigate the role of *space* in psychotherapy and cognition together, stems from MSP its main hypothesis. Which is, that spatial cognition is the foundation of all – no matter how complex, abstract, problematic, symbolic, concrete or imaginative – thought. In other words, the most basic embodied spatial orientation skills, support (scaffold or ground) everything else that is going on in peoples' minds (Spivey, Richardson & Zednik, 2010; Barsalou, 2012). The logical link to clinical work comes from the observation that in several psychotherapeutic traditions, like “psychodrama” (Moreno, 1951), “family constellations” (Weber, 1994) “the personal timeline” (James & Woodsmall, 1986) and “the social panorama” (Derks, 1996) *space* is in fact the central mode of operation (Derks, Walker & Ötsch, 2003).

Space in psychotherapy

The use of space in psychotherapy means either that,

- 1) The therapeutic process is choreographed in a 2-D or 3-D manner, and/or
- 2) That the therapy focuses on the locations where the clients' limiting concepts are projected (spatially indexed, visualized, heard or felt) in order to move them towards more favorable places.

That means that the client either physically moves (walks) along marked spots on the floor (or between seats) that symbolize certain personal meanings to him/her, or that he/she explores where in his/her imagination certain critical concepts are seen, felt or heard in order to mentally move these to better sites.



Spatially choreographed therapy



Spotting the location of concepts

The primary reason to explore spatial psychotherapies is their supposed strong effect. This goes so far that some say that, whenever space is included in psychotherapy, miracles start to happen (Grove, 1998; Lawley and Tompkins, 2000, 2002, 2014; Battino; 2006; Hoyt & Talmon, 2014). *Miracles* here means that, clients that have walked around for years with serious issues find relief within one single two hours session. Thus the logic behind this project is: That when we assume that spatial psychotherapeutic interventions have indeed such a “miraculous” impact on peoples minds, this must point at not yet fully understood principles. But what kind of principle are we talking about?

0.1 Hypothesis

The hypothesis used in this study parallels the view of researchers like Tversky (2004), Barcalou (2012), Spivey, Richardson & Zednik (2010),

which is that space must be the primary organizing principle in the mind. In other words, *where* thoughts/images/feelings/sounds are projected in the psychological sphere in and around a person is what makes them retrievable, understandable, distinctive, and gives them for a large part their cognitive-emotional meaning. In brief: all cognition is spatial in nature.

The probable reason why this is the case, is, that the same spatial cognition that an organism uses to orient itself in the physical world (to know where its body is located in the environment) is build upon by all other mental functions (Lakoff & Johnson, 1999; Bergen, 2012; Bailey, 1997; Tversky, 1991, 2002, 2010; Barcalou, 2012; Burgess, 2014). Thus, how we create an embodied simulation of ourselves in our physical environment in our mind is similar to how we map everything else up to the highest levels of abstraction: in our mental space.

The greatest challenge in regards to this hypothesis is that, what is going on in mental space stays largely outside of awareness: it functions unconscious. But this unconscious character doesn't make it less fundamental! Luckily, we have reached an age in which psychology can deal with unconscious processes (Zajonc, 1984; Bargh, Chen & Burrows, 1996; Kahnemann, 2013).

Spatial psychotherapies

When we mention *spatial psychotherapies* as one single category, we refer to a class of methods that at first sight may not appear related. And their commonalities are frequently diffused on purpose since, competition makes that their proponents primarily emphasize the differences.

To name some of these methods: The Other Minds Eye (Sargent, 1999), The Feldenkrais Method (Feldenkrais, 1972), Voice Dialogue (Stone & Stone, 1980), Spatially sorted NLP (Andreas and Andreas, 1989; Dilts, 1990; Andreas, Faulkner & McDonald, 1994), Time Line Therapy (James & Woodsmall, 1988), mBIT (McCraty 2003), Clean Space (Lawley & Tompkins, 2006), Family Constellations (Weber, 1994; Hellinger, 1995; Hellinger & Beaumont, 1998), Structural Constellation Work (Varga von Kibèt, 1995; Varga von Kibèt & Sparrer, 1998), BrainSpotting (Corrigan & Grand, 2013), Time-Lining, The Matrix Model and Semantic Space (Hall & Bodenhammer, 2001), Acceptance and Commitment Therapy (Hayes, Luoma, Bond, Masuda & Lillis, 2006), The Wholeness Model (Andreas, 2014) Metaphors of Movement (Austin, 2010), The Social Panorama (Derks, 1996) and we must also include a great diversity of Shamanistic psychotherapies, Ayurvedic psychotherapy, Chakra/Aura/spatial-energy and Holistic psychotherapies. Again, the reader must take notice of the fact

that, many founders of-, researchers of- and practitioners of these schools, almost never mention their method has its use of space as a central principle.

0.2 Phenomena in Mental Space

In their quest for structure, MSP researchers (Derks, Ötsch & Walker, 2011) came to distinguish 10 spatial domains of study:

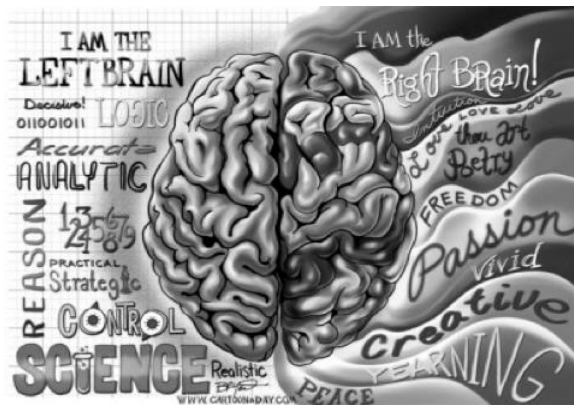
- 1) The brain itself is a physical 3-D structure.
- 2) The central nervous system is also a physical 3-D structure.
- 3) The body is a physical 3-D structure too.
- 4) The awareness of percepts, memories, emotions and concepts appears in an infinite 3-D sphere in and around the person (often just called *mental space*). Within this “awareness space” we find six sub-categories:
 - a) Bodily whereabouts: navigation and orientation.
 - b) Time as a spatial construction.
 - c) The self, as a spatial construction in and around the body.
 - d) The social world as a panoramic 3-D landscape.
 - e) All linguistic meaning appearing somewhere in mental space.
 - f) Spatial metaphors: this is the use of 3-D situations as analogies for (emotional) states of affairs (problems).

1.0 Brain Space.

It had been in neuro-psychology, where one works with patients with disrupted brains, where the significance of the brain having two hemispheres,

that both have a different way of thinking, has been a central focus (Gazzaniga, 1970; Bishop, 2014).

In the 1970s the cognitive differences between the hemispheres were the focus of research (Gur & Gur & Marshalek, 1975). All human behavior and cognition could in some way be related to these phenomena. No area of cognition was left out. Everything people did was evaluated on how this related to the division of labor between the hemispheres (Bakan, 1978). The whole idea fascinated the audience and became hype in popular science. In the 1980s the workings of many new-age psychotherapies were explained by how such a method activated the more creative right hemisphere or helped to improve the connection between the two sides of the brain, to stimulate more integrated thinking.



The hemispheres in the 1980s

After the interest among psychotherapists had waned off in the 90s, a clear practical tool for using the difference in cognitive activity in both hemispheres was developed by Allen C. Sargent (1999) and formulated in a book entitled: “The Other Mind's Eye: The Gateway to the Hidden Treasures of Your Mind.”

Sargent gives the following steps as an outline for his technique:

Step 1 - The first step is the understanding of the concept of the External Dominant Eye. If you look at a spot about 20 feet away with your head facing straight, focus on a spot and simply point at it from the center line of your body. Close one eye and notice if your finger is still directly on the spot. Switch back and forth from one eye to the other and then open both eyes. Can you see the difference?

Step 2 - Get an internal picture of a person or event.

Step 3 - Note the specific qualities of the image - Visual, Auditory, Kinesthetic and Emotional.

Step 4 - Notice or get a sense of which internal eye you are using to see this picture.

Step 5 - Now, in a way that works for you, shift so that you are now seeing that person

or event with your other internal mind's eye.

Step 6 - Note any changes in the way this second image is represented - Visual, Auditory, Kinesthetic and Emotional.

Clinical experiment 1: Switch Hemisphere

- 1) Find a memory or a future situation (an issue) that when thought of raises negative emotions and think of it until these emotions are clearly sensed.
- 2) Imagine that your mind is a car. When you think of the emotional issue, are you sitting behind the wheel or in the passengers seat?
- 3) When you believe you know from what side you are thinking of the issue, then do the next: Imagine to open the door of the car (brain) get out and walk along the back side to the other side of the car (brain) and then get in and sit down at the other side.
- 4) Now think of the issue from the other side of the car (brain).
- 5) Notice the difference in the quality and intensity of the emotions.

2.0 Central Nervous System Space

The central nervous system is a 3-D spatial structure that connects the brain with the body its organs and senses. Over the last 40 years, neurological

research revealed great populations of neurons around the gut (Gershon, 1998) and the heart (Waytz, 2010). Some have suggested that these cells are similar to those found in the brain but others deny that.

These findings gave way to speculations about the so-called second- and third brains, and intelligent processes taking place in and around these organs. The supporters of this view, point at the natural language (but also at poetry) where they found countless examples in which the gut and the heart are portrayed as important centers of experience and cognition. Until recently, most neuroscientists have taken these expressions as metaphorical: as a manner of speech that described processes that in reality are brain based. Now the idea of intelligence housed around the major organs was seen as probable (Waytz, 2010).

“mBIT” is a therapeutic approach that stands for Multiple Brain Integration Techniques: the multiple brains are that of the heart, the head and the gut (Oka & Soosalu, 2012). “mBit” is building on the classic NLP-skills and makes use of a breathing method when it comes to accessing the information from the Cephalic Brain (head), the Cardiac Brain (heart) and the Enteric Brain (gut) separately (McCraty 2003).

The gut-brain is said to deal with identity, self-preservation and mobilization (Oka & Soosalu, 2012). That also means that courage is something that stems from the gut, which matches popular expressions. Logically the gut intelligence might be an extension of digestion of which other experts say is a highly intelligent neuro-biochemical process. Thus if so, then “liking” and “disliking” might as well be its major output (Gigerenzer, 2008).

In the natural language we can hear that the gut and the heart express themselves over the kinesthetic channel in the shape of heart- or gut feelings. In the therapeutic practice these feelings are a recurring phenomenon: recently some psychotherapist started to take the intuitions that the central nervous system signaled over its main organs more serious.

Psychotherapeutic interventions that aim exclusively at the heart can be found under the name Heart Coherence (McCraty, 2003; see also HeartMath Research Center). In this method breathing is used to regulate the heart rate and its pumping quality. The client must pay attention to this organ while inhaling and exhaling in a prescribed manner. Also biofeedback tools are used to improve the clients’ influence over his autonomic nervous system. Heart Coherence is also used as a psychotherapeutic “something completely different” on its own – and used as such, it might be comparable to meditation trainings like Zen, mindfulness, or self-hypnosis, where concentrated breathing brings the client in an altered state of consciousness that, by itself alone increase the

chances of resolving personal issues (Lebois, Papies, Gopinath, Cabanban, Quigley, Krishnamurthy, Barrett & Barsalou (2015).

The relevance of higher cognitive activity and the connection, disconnection or conflict between head, heart and gut, is best known from decision-making and dilemmas: “My head says yes but my heart says no.”

Over the last decades it is not uncommon to have psychotherapy clients communicate with their younger or older selves, immune systems (Dilts, 1990), hormones, cancer cells, breasts, ovaries, leucocytes (Spiegel & Bloom, 1983). Thus psychotherapy with “parts” may work probably just as well when the symbols chosen are called brain, heart and gut (Durlinger, 2009). But the big question is, can one isolate the intelligence of a single organ and make use of its knowledge separately from that of other parts of the central nervous system?

Clinical experiment 2: Head, heart and gut.

- 1) Think of a dilemma about something that must be decided. Like buying a new item, choosing a holiday destination or quitting ones job or partner.
- 2) Create relaxation over your breathing rate and breathing depth.
- 3) Now formulate only one side of the dilemma as a clear positive statement: “I will do X”.
- 4) Now put your dominant hand on your heart and repeat the statement while sensing your heart feelings about it. Is your heart in full agreement or is there some doubt? Scale the measure of doubt felt on a 5 points scale. 0 = fully disagree.... 2, 3, 4, 5 = fully agree.
- 5) Now put your dominant hand on your forehead and repeat the statement while sensing your head about it. Is your head in full agreement or is there doubt? Again use a 5 points scale.
- 6) Now put your dominant hand on your belly and repeat statement while sensing your gut feelings about it. Is your gut in full agreement or is there doubt? Again use a 5 point scale.
- 7) Now explore the content of the doubt of either center.

3.0 Body Space.

The body is a three-dimensional structure in which the 3-D nervous system connects everything to a 3-D brain in such a way that the body can be moved around in the 3-D universe. With the help of Antonio Dimásio's "*Descartes' Error: Emotion, Reason, and the Human Brain*." (1994), the link between brain and body, and its implications for cognition were drawn to the fore in neurology, psychology and philosophy. The so-called "Embodied Cognition Theory" (Lakoff & Johnson, 1999; Bergen, 2012) states that bodily experience forms the basis for the development of all cognitive functions. This means that all we know is learned on top of us becoming capable of nourishing and moving our body through space. Basic body experiences in the womb and early childhood become the foundation for even the most sophisticated cognitive strategies. For instance, finding our balance in early childhood can be the prototype for a balanced life style in adulthood (Lakoff & Johnson, 1999).

Clinical experiment 3: Writing in Space.

This experiment was re-designed by the Canadian Feldenkrais practitioner David Keatz in 2015. It is important to know that most Feldenkrais exercises are done on the floor. This redesign makes it usable in a sitting and standing position.



David Keatz

Preparation:

- 1) Sit on the tip of your seat. Draw slowly 4, 20 centimeters wide circles with your nose in the air. First clockwise and then anti-clockwise.
- 2) Next, write your name with your nose in the air with the letters on the same spot in space.
- 3) Pick a psychological issue and name it with one single word.

Continuation:

- 4) Open yourself for experiencing the negative feelings that belong to this issue: step into the problem state.
- 5) Sit on the tip of your seat and put both your hands, fingers locked, behind the back of your head with your elbows pointing forwards.
- 6) Write the word by which you have named the issue in the sky with both elbows (as parallel pens) pointing forwards.
- 7) Write it with large letters. Do the same again but very slow. Do it one more time standing and also with an even larger letter-height (of 50 centimeters high). Repeat this twice and very slow.
- 8) Now observe while you do it again for the 3rd time, where you experience any kind of obstruction in your movement. Then focus on these obstructions.

Evaluation:

- 9) What do these obstructions in movement tell you about your psychological issue?
- 10) Test the effect of this exercise on the intensity of the feelings about your issue.

4.0 Awareness Space.

What we call here “awareness space” is a sphere that starts within the body that theoretically extends into infinity and that is used to represent a model of the world. In other words: since reality is 3-D the mind simulates it in the same manner. Thus the imaginary space in which reality is re-created, is what we call “awareness space” and it functions as the 3-D blackboard of the mind.

The main problem with awareness space is that what is represented there, stays largely below the threshold of consciousness. Thus, people are only slightly aware of the exact content of awareness space. Researchers can only gather information about the objects in awareness space by means of indirect methods: make the participant respond to questions that can only be answered on the base of the “unconscious”, “intuitive” or “background” knowledge stored in awareness space. For instance: “Point at where you think of your mother.

The processes by which a real object becomes represented at a location in awareness space, is called “mental spatial indexing” (MSI).

“Spatial indexing,” is a term from computer science, that describes a way to create a human/computer interface. In Wikipedia we read:

Spatial indices are used by spatial databases (databases which store information related to objects in space) to optimize spatial queries. Conventional index types do not efficiently handle spatial queries such as how far two points differ, or whether points fall within a spatial area of interest.

Most present day computer operating systems can make use of that (Google Earth, Google Maps, Apple Maps). In the best examples a collection of icons is represented in a 2 or 3 dimensional way on the screen. When the user clicks on the icon, or in the case of a touch screen, touches the icon, the files of information that are indexed there become available.

In the mental-space-psychological form of spatial indexing the “icons” are often kept below the threshold of consciousness, but they do exist in the form of some kind of mental images. It is by gazing at the location or gesturing in the direction of these unconscious images that the information stored in the neural tissue becomes activated in the mind’s “working memory”.

The verbal and non-verbal behavior underlying the activity of spatial representation (the storage and retrieval of information on the base of locations in mental space), was described in detail by Barbara Tversky in a series of studies and overviews dating from 1991 to 2008. She created the term “spaction” for spatial actions that coincide spatial cognitive processes. Tversky (2004) developed a great understanding of the unconscious qualities of what is represented in mental space:

Mental models contrast with classic images in that they are more schematic than classical images. Entities are represented as tokens, not as likenesses, and

spatial relations are approximate, almost qualitative. A similar view was developed to account for understanding text and discourse, that listeners and readers construct schematic models of the situations described (Kintsch & van Dijk, 1983; Zwaan & Radvansky, 1998). As is seen, visuo-spatial mental representations of environments, devices, and processes are often schematic, even distorted, rather than detailed and accurate internalized perceptions. (p. 212)

In the previous chapter we already implicitly mentioned the role of “mental spatial indexing” in psychotherapy. There are good reasons to believe that the awareness of how people access information over MSI must date back to shamanistic practices from the stone age. In modern times its therapeutic use was first described by NLP-therapists (Bandler, 1986; Derks, 1997; Hallbom, 1995) who found that their clients focused their eyes at certain locations in mental space while they explored particular (traumatic) memories. Before that, Bandler & Grinder (1979) had already observed that memory retrieval coincided with eye movements, that they associated with the getting access to information in different sensory modalities. However, when researchers could not verify Bandler & Grinders’ (1979) so called “eye accessing cues” (Heap, 1986), this helped to discredit NLP in the circles of academic clinical psychology up to the current date. At about the same time as Bandlers’ general approach to sensory qualities in imagination came to the fore, James & Woodsmall (1986) described the particular case of the subjective construction of time. The spatial qualities they paced central, as a linear structure: The Personal Time Line. In NLP this became the prototype for the clinical exploration of awareness space (Derks, 1994). Besides the “eye accessing cues,” Bandler and Grinder and their co-developers of NLP also recognized the nonverbal behavior of gazing and pointing and gesturing as signs of memory retrieval. In the work with time, it was people that were “looking to the past” or “staring at the future” that helped therapist to notice the signs of “mental spatial indexing” (Jacobson, 1986).

To understand the non-verbal signs of MSI, one needs to see this in the light of how it may differ along with the state of consciousness of the subject. A fully awake subject will do the classical NLP-eye accessing cues including pointing and gesturing, while they retrieve information. But along with an increasing relaxation the pointing and gesturing will be reduced to finely stop. In case of a deep relaxation (deep hypnotic trance) no other signs can be seen then saccadic eye movements during a forwards stare.

Logically speaking, we must distinguish between the moment of storage in mental spatial indexing and the moment of retrieval. In the non-verbal behavior of therapy subjects both of these moments are followed by quite similar markers: like nodding and saying “Yes” or “Okay” or an equivalent of that. Also the therapist use a similar non-verbal behavior when their client points out the location of some event in space for them: Okay, thus primary school is over there...”.

All in all this resulted in the idea that clients (people in general) carried their model of the world with them as a landscape of mental spatial indexed information. Hall & Bodenhammer (1999) started to call this “the matrix” or “semantic space”. Derks (2002) explored the social aspect of MSI, in his Social Panorama Model, in which the locations of the representations of people are the center of attention. From the perspective of mental spatial indexing, the representation of a person (called personification, Derks, 2002, 2006) is a file of knowledge and assumptions about that person in which the image – that is often no more than a caricature – functions as the icon; by looking or gesturing towards this unconscious icon the information about a person can be activated.

From all the above explorations came the ruling idea in mental space psychology, which is, that all that is happening in the mind is happening “somewhere”. But exactly this very principle that, everything people are aware of has a location was long overlooked in the history of psychology (Jaynes, 1976). And then, after this view was finely acquired, it took a turn in paradigm to grasp its implications for psychological functioning (Walker, Derks & Ötsch, 2009).

As already stated above, the explanation for the coming into being of “awareness space” is, that, everything in the universe is located somewhere and that the psyche automatically simulates this spatial nature out of the necessity for the organism to survive within its 3-D environment.

When regarded this way, mental space is primarily a neuronal copy of physical space. At the start of the development of an organism, real objects are probably only represented on the locations where they appeared most in physical space and then relative to the position of the organism (Milner, 1991). Repeated perception of the same real objects on the same spots leads to generalized ideas about the steady-ness of the locations of familiar objects (Derks, Ötsch & Walker, 2014). In other words, where things are located in reality leads the way to where they are represented in the mind. However, the mapping of the locations of real objects can be extended to similar mappings of abstractions and mental constructs that have little or no roots in physical reality (Tverky, 2010). In other words, the spatial representation of reality can be extended to a similar representation of generalizations, fantasies and abstract images (Derks, Ötsch & Walker, 2014).

“Where” an object is represented in mental space has great implications for an organism: it enables it to navigate in order to have food, shelter and partners (Dolins & Mitchell, 2010). To represent things on a mental (mental spatial indexed) location helps to find stuff, and it has the same function and priority for the representations of non-physical items in the mind. And there are good reasons to believe that a large part of the emotional meaning of

“something,” is encoded in the spot where this “something” is mental spatially indexed (Derks, 2006).

Clinical Experiment 4a: An emotional neutral object

- 1) Pick some emotional neutral item in your mind: like maybe a baking glove, a banana or a sock.
- 2) Find the location in your mental space where you think of this particular item.
- 3) Now move the image of the item into the center of your visual field; move it to 15 centimeters in front of your nose and enlarge it to 3 times its real size.
- 4) Evaluate the emotional impact of these steps.

Wolfgang Walker (2006) studied the spatial coping patterns in traumatized clients. Their symptoms often have to do with how the incomplete spatial coping has disrupted the coherence in their mental space. Their awareness space becomes split up and broken at the cost of the unity in their personality and also their experience of the continuity of time – which is connected to their life history (Hilgard, 1977). Beside that it may lead to a disruption of their image of the social world too, causing them to maintain problematic relationships which appear like “bi-locations” in their social panoramas (Derks, 2002).

Forms of incomplete spatial coping:

- 1) Putting it at distance,
- 2) Distancing oneself,
- 3) Stepping out of oneself.

Putting it at distance is the least pathological way of incomplete spatial coping since it leaves the position of the person in mental space intact. It helps to reduce the limiting effect of arousing stimuli on the creativity of the person. To get away from the “emotional noise” can be a good step towards problem solving. That is why “taking distance” has been used as a sort of cornerstone “something completely different” in many types of psychotherapy like NLP, any kind of Imagination Therapy, Mindfulness and also in oriental traditions.

Stepping out of oneself and taking on the observers position, (camera position in RET, visual-kinesthetic dissociation in NLP), is also a type of spatial “something completely different” and it has proven to be highly effective in bringing problems closer to their conclusion. But it can only be used for a permanent resolution if the distancing from the self is only taken on during the therapeutic intervention as a part of the techniques used. Thus, in the end the

client must get back into his original “self position” and become capable of dealing with the (traumatic) issue from direct confrontation.

4.3 To where in mental space are you looking?

The moment of origin of spatial psychotherapies, in which mental spatial indexing is the central tool, lies in the middle of the 1980s. It was when Richard Bandler (1985) introduced his concept of “sub-modalities”: he analyzed the sensory qualities of the experience of his client in more detail than anyone had done before. Among these sensory qualities “location” stood out as very influential. Thus for the development of mental space psychology this was a milestone. We already mentioned that, to where in mental space a client keeps their eyes focused (stares) at the moment he or she thinks of something (problematical) helped to discover the phenomenon of “mental spatial indexing”. This staring at some critical location in mental space, was also discovered by the Dutch therapist Henk Hoenderdos (1994), who called it “Focal-Points”. The application of mental spatial indexing in trauma therapy is also present in a technique called “Synesthesia Pattern” developed by Tim Hallbom (1995). And currently has found its way from Neuro-linguistic Programming to the more academic style of psychotherapy that is an offshoot of EDMR (Shapiro, 1995) called “Brainspotting” (Corrigan and Grand, 2013) – a method already mentioned when we discussed its neurological underpinnings.

The skill of noticing the point where a client fixates his eyes, at the exact moment this person is emotionally connected to the problem state, has proven to be a very useful skill for therapists. Once this fixation point is noticed, the location of the problematic image in mental space can be defined. However, the same also holds for finding the ideas that are helpful for solving the problems: resources.

Clinical experiment 4b: moving an emotional laden image.

- 1) Think of something that is worrying you.
- 2) When you experience the feeling of worrying, then explore with your partner: Where do you visualize the image that makes you worry?
- 3) Have your partner spot the exact location (distance, direction, elevation and size) of the image in mental space.
- 4) Next your partner helps you to find a better location for this image. This is done by hand gestures: the image is moved by “grabbing” it first and then taking it further away or closer, from the left to the right or from below to higher up.
- 5) The intensity of the sensed feeling of worrying is what guides the movement until a better location is reached.

- 6) The question is: can the image stay there? Or is something important lost that way?

From where in mental space are you looking?

The sphere of awareness-space extends around a person's body – logically one would expect the experience of the self to be located in its center with the rest of the world being regarded from there as a panoramic landscape. However, it is not only the occurrence of the so called “out of the body experiences” (Mavromantis, 1987; Marselli, 2015) that shows it is not always that simple.

Based on Ramana Maharshi and oriental philosophies, meditation practice and aided with the NLP concepts of core states (Andreas & Andreas, 1994), sub-modalities (Bandler, 1986; Andreas & Andreas, 1989) and perceptual positions (Cameron-Bandler, 1984; Grinder & DeLozier, 1987), Connirae Andreas developed a procedure she called the Wholeness Process (Andreas, 2015). The spatial phenomena Andreas works with in this approach are the locations *from where* a person is aware of critical concepts. She calls these spatial positions “I” locations.



Connirae Andreas

A metaphor for the “I” positions in Andreas’ work can be found in filmmaking. An “I” position is like a camera angle or camera position. During the shooting of a movie scene, more cameras can be used simultaneously to capture the events from different angles. In movies, cameras normally are placed in such a way that they do not have each other in sight. But in awareness space, the “I” positions do exactly that: they look upon each other. In fact these “I” cameras are positioned in such a way that the one focuses on the other and that one on the next and so on. It is like a series of cameras that are filming one another at the same moment in time.

In Andreas work, the “I” positions are generally found within, around and at greater distance from the body. The core of the method comes from noticing that the “I” positions are sustaining each other and by that also consolidate

certain problem related concepts. Thus a problem state is kept steady by means of a series of interconnected “I” positions.

“I” positions are probably linked with each other in a way that follows a person’s cognitive and emotional development. A chain of such “I” positions can hold a problematic concept stable and steady in the mind. By special questioning techniques such a chain of “I” positions can be followed back from the problem concept to its origin. The search follows a structure of locations that, at times may zigzag through mental space. The special therapeutic effect of the Wholeness technique arrives from a major discovery: That the first (or earliest or oldest) “I” position is in itself not consolidated by other “I” positions (earlier ones) and is very open to let go its boundaries. The instruction for this transformation comes as a sort of invitation to this primary “I” position to relax and dissolve in the greater field of awareness.

The therapeutic quality of the Wholeness Model stems from using the unique property of chains of “I” positions: they can be dissolved from their earliest origin over the intermediate “I” positions to the present problem concepts. Metaphorically it works like a line of Roman foot soldiers that cover each other’s backs. When you attack them from the front, they are unbeatable. However, if you first attack to last one in the row, the rear man, who has no one to back him up, he will be easily concurred. Then the next one in line becomes just as vulnerable without a back up and can be subdued with ease. Then the entire line of warriors up to the front man (the problem concept) may be beaten that way. This subtle form of “something completely different” of the Wholeness process helps to free the person from the rigidity that underlie his or her problems.

Clinical Experiment 5: Unlocking “I” positions.

- 1) Think of a psychological issue that is limiting you. Make it your actual experience by stepping into the state of having the issue. Make sure you feel the uneasy feelings that go along with that. Explore the sensory qualities of this experience. Make sure that you know exactly where these sensations are located in mental space.
- 2) Now explore from which position, spot, location you are aware of this issue and its sensations?
- 3) Explore the sensory qualities of this spot. We call it the first “I” location.
- 4) Now explore from which position, spot, location you are aware of this first “I” location?
- 5) Explore the sensory qualities of this spot. We call it the second “I” location.
- 6) Now explore from which position, spot, location you are aware of this second “I” location?

- 7) Repeat this process until there are no new “I” location found.
- 8) Invite the last found “I” location to relax and dissolve in the wide field of awareness. Give this some time.
- 9) When the last found “I” location is dissolved, do the same with the previously found “I” location. And continue this until all “I” locations are dissolved including the location of the problem.
- 10) Explore the impact of this process.

4.5 Depression in mental space

When Andreas (2014) introduced the Wholeness process, she presented it in the context of oriental meditation. She also brought it as a way to get rid of *the ego* and to become *enlightened*. From the NLP-tradition of listening in a literal manner to such expressions, one would expect that “enlightenment” is an experience in which ones mental space lightens up.

In contrast, the experience of depression is always associated with darkness. Psychologists tend to see the link between *depression* and *darkness* as a metaphor. It is the same with “enlightenment” in which the *light* is seen as an analogy for euphoric or clear states of mind. However, clinical practice hints to the existence of more literal forms of darkness and light. From clinical work we know, that clients may spontaneously say that “all became lighter” after successful therapeutic interventions.

After working with clients with mood disorders for several years the hypothesis came up, that a sense of darkness appears to be a reliable subjective epiphenomenon of depression. When asked for it, clients can point out where such dark areas are located. In general these are found close to the body, on all sides except the center of attention: not in front of the client’s eyes.

A next discovery is, that this darkness signals repression. Since most clients with dark zones seemed to have repressed life-issue that they cannot handle and feel helpless about. In working with the clients clouds of darkness it became evident, that only becoming aware of such clouds has no therapeutic value. It is what is hidden behind the darkness, the repressed issue that needs to be the real target of therapy.

Mood disorders, such as depression and bipolar disorder, are a leading cause of disability, with 32 percent of Americans suffering from these disorders within their lifetime. The suicide rate among people with mental disorders like depression is 30 times higher than the rate among the general population.

Among psychiatrists, the discussion about the treatment of depression centers round antidepressant drugs versus talking therapies. However, since drugs work quite reliable and easy, and are promoted by the pharma-industry, most of the discussions (in the Psychiatric Times) are often concentrated on how to know what dose or mixture will work for a particular type of patient. In general monoamine oxidase inhibitors, tricyclic antidepressants or selective serotonin reuptake inhibitors are the weapons of choice.

Due to the systematic over representation of evaluation studies which show positive therapy results, a too sunny idea of therapeutic effectiveness is created. This holds for both, psychotherapeutic and pharmaceutical treatments of depression. That is why Fournier, DeRubeis, Amsterdam, Shelton & Hollon, (2016) believe that the debate about what works best cannot be concluded. And Steven Hollon, a professor of psychology at Vanderbilt University comments:

Both talk therapy and antidepressant drugs are efficacious. They're just not as efficacious as we think they are.

A central problem in the psychotherapeutic treatment of depression is the widespread belief that depression is mainly a neuro-physiological disorder that needs a biochemical solution, for which there seems also much support from experimental studies. Some of this evidence is ambiguous. For instance, the so-called serotonin transporter gene seems to play a role in depression. When this gene was shorter than regular, this was associated with an increased risk for depression, but only *if the bearer experienced adverse childhood events* (Caspi, 2003). Thus when the client had a traumatic youth the shorter genes might express themselves in the form of depression. Benedetti (2014) confirmed this also with patients who were diagnosed with bipolar disorder. The problem with the latter group is that they respond only little to the anti depressant drugs that were mentioned above, but they respond better to lithium. While researchers have found that other non-lithium based drugs are still effective at managing certain symptoms of bipolar disorder and depression, they are not very effective at preventing suicidal thoughts.

Lithium, however, was found to be an effective anti suicidal medication for patients with bipolar disorder or depression. Although the two disorders have different symptoms, the underlying depression connects them. The effects of lithium can be an uplifting of the patients mood but it may be damaging to the liver. Benedetti (2014) found that sleep deprivation and light therapy reduced the suicidality among bipolar depression patients often rapidly. Which is a great finding, since one third of bipolar patients in a depressive phase do suicide attempts.

The severity and the long duration of depressive symptoms is another stand in the way for psychotherapy: doctors, psychiatrists and clients alike often do have a low expectation of “talking therapies”.

All in all, this makes a very brief therapy for depression something hard to sell: most people cannot believe that it can work that fast and easy. And a therapist who proposes the possibility of a brief anti depression psychotherapy is not likely to be taken serious. However... it will be a single session spatial psychotherapy approach against depression that we will look at in this section.

4.6 Repression, a Freudian fossil or real?

A great candidate to become a law of mental space psychology is, that, when a mental object (image, feeling, memory, etc) is moved away from the center of attention (straight in front of the nose) it becomes less conscious (Walker, 2014). That means that people can move (MSI) hard-to-deal-with ideas to the sideways or even to locations behind them, and by doing so they will not

be bothered by them that much. We already mentioned that this leads to forms of incomplete coping: since the client still cannot deal with the problem stimuli.

In addition people can quit thinking of something by training their selective attention to stop the idea before it comes to awareness: that is what Freud called repression. But this may come at a price, as most clinical psychologists and psychiatrists know since the work of Pierre Janet (1889) at the end of the eighteenth century. In the long run such “repression” (Singer, 1990) may cause a great variety of symptoms of which many can be called “depression”.

If a depression stems from the client repressing an area of life they cannot cope with, there are some brief technical steps that can bring relief. The procedure as presented below is quick and easy; this is at the same time its greatest disadvantage. Since the amount of suffering (15 - 25 years?) and the easiness of the ritual that brings its resolution is way out of balance; which may keep clients from accepting this as a genuine treatment (Hollander, 1995).

Extra experiment: Depression in Awareness Space

First phase

- 1) Ask the client to sense the feeling of sadness that typifies their depression.
- 2) When the client signals that the feeling of sadness is present, ask the following presupposing question: “Where in the space in and around you do you sense an area of darkness?”
- 3) Let the client point out the exact location of the dark area.
- 4) Next, suggest that the client imagines that the sun rises somewhere behind him or her, in such a way that it can shine on the dark area.
- 5) Suggest that the client moves the dark area in the direction of the center of attention. In front of their eyes.
- 6) Now give the client the time (10 minutes) and rest to do all of this and to also witness what happens. Let the client tell about it.
- 7) One can expect the dark area to slowly shrink or melt and become significantly lighter. This is something to give time.
- 8) Next; ask the client to find out what the issue is that was hidden behind the dark area: this is probably something they could not deal (cope) with. Give the client enough time to explore this.

Second phase.

Until present, the author and some of his colleagues were very successful by continuing this process with the aid of a classical NLP-technique: The use of an inner model. The question to the client is: Do you know somebody, or do you believe someone exists, that is able to deal with the issue that was hiding behind

the dark area? This example is used to find the appropriate response to the issue. If what the model is imagined to do appears right, the client tests it first in a dissociated form to next step into it. The client needs to be congruently sure about the appropriateness of the new behavior and coping method. If not, the inner model can be explored and copied again, or other such models can be used for the same purpose.

An extensive test of the ecology of this new behavior is required.

Caution:

It is not helpful to only do the first phase without helping the client to find the appropriate behavior, because then the client is only aware of his or her failure to deal with some part of life (Krakauer, 2000).

5.0 Navigation and Orientation.

That spatial cognition, and all animals' navigation skills, builds on early body awareness is now generally accepted in the field of spatial cognition. However, that the same skills of spatial orientation can help a client in therapy to sort out his or her problems is not so widely recognized (Derks, 2014). This comes because only few researchers in this field are familiar with spatial psychotherapies. But the link becomes easy to grasp, when one first sees how spatial orientation forms the basis of all cognitive skills. When we make the paradigmatic step to assume that everything in the mind is spatially organized, then psychological problems and their solutions cannot be an exception to that (Derks, Ötsch & Walker, 2014).

Clinical Experiment 6: Increase the level of spatial structure.

When you lead the participant through this experiment, you should not give any suggestion for a solution whatsoever, during the entire experiment.

- 1) Format 1. Start a conversation with someone about his or her *desired outcome*. Also explore, the *present state* and the emotional *obstacles* that keep the person from achieving his/her *desired outcome*. Do this for 10 minutes. Notice that this conversation centers around 3 distinct themes: Desired Outcome, Present State and Emotional Obstacles.
- 2) Format 2. Now, take three sheets of paper, write DO (Desired Outcome) on one and PS (Present State) and EO (Emotional Obstacles) on the other ones and put these on the table. Include these sheets of paper as symbols in the conversation by pointing at them at relevant moments. Continue for at least 10 more minutes.
- 3) Format 3. Now, put the sheets of paper on the floor and have the person organize these papers spatially. Make the person step on one at the time according to what is spoken about. For instance, when there is spoken of the *Desired Outcome* (DO) this is the sheet to stand upon. Do this again for 10 minutes.
- 4) Format 4. Now, let the client speak about his/her issue from standing on one of the sheets of paper and address what is said to the relevant other sheet according to the content at the time. For instance, from the *desired outcome* the person may speak to the sheet that represents the *emotional obstacles*. Do this also for 10 minutes.
- 5) Format 5. Now help the client to visualize him or herself as being exclusively the part that is the DO. Then step into this position by stepping on the sheet. Help the person to become emotionally connected to DO when standing on the sheet. Do the same with PS or the EO. Again for 10 minutes.

Clinical Experiment 7: The Clean Space Experiment

(Based on James Lawley and Penny Tompkins, 2003)

1. The client thinks about a theme that is worrying him or her until he gets stuck in negative feelings (problem state).
2. First Clean Space question: Where (with a gesture around the room) is a place that knows more about this theme?
3. The client will indicate a place in the room. The guide encourages the client to go to that place and to open himself up to whatever the place knows about the theme.
4. Second Clean Space question: What is it that this place knows about the theme?
5. The client thinks about the theme from that place until he gets stuck in negative feelings again. (next problem state)
6. First Clean Space question: Where (with a gesture around the room) is a place that knows more about this theme?
7. The client indicates another place in the room. The guide encourages the client to go to that place and to open himself up to whatever the place knows about the theme.
8. Second Clean Space question: What is it that this place knows about the theme?
9. These steps are repeated until the client no longer gets stuck in negative feelings.
10. Then go back to the start position and retrace all the steps that have been made.

6.0 Time/Space

Most philosophers that studied the concept of time realized quickly that it must be a product of space: and to be precise, a product of the movement in space in relation to points of reference (Kant, 1781). Only objects that change their position in space create the suggestion of the passing of time. These can be orbiting heavenly bodies, the movement of the hands of a clock, revolving sub-atomic particles or any other steady movement in the natural world.

Whyatt Woodsmall and Tad James (1988) published “Time Line Therapy”. This approach earns to be called “the prototype of spatial therapies” and it can be used for two distinct psychotherapeutic activities:

1) The application of a line-like image that represents a person’s course of life. When represented on the floor, therapist will use sheets of paper that symbolize the moment of birth and the here and now. This way of working is called “The Line of Life” (but many call it incorrectly “time line”). A variety of therapeutic operations can be given shape on the base of The Line of Life and its value in therapy cannot be overestimated. It delivers an extension of all variations of Change Personal History (Bandler & Grinder, 1979; Isert & Rentel, 2000) and provides them with a more robust spatial structure.

2) The other therapeutic activity in Time line Therapy consists of the analysis and adjustment of the spatial model of time that a person has constructed in the course of their life. This so-called “Personal Time Line” is seen as the primary factor deciding over how a person deals with time. Any problem with motivation, planning, stress, with being in the present or not, but also forms of future or past related depression, can be related to how a person has constructed time. For instance, a client of the author had *the past* in two different locations in his mental space. This implies in MSP, that he experiences two distinguishable pieces of past. The one piece was strait behind him, and went from his birth to age eight. That was the moment he had been kidnapped by his father after his parents divorced. The past from age eight up to the present was located at the 10:30 O’clock direction in his mental space – thus in front of him left of the center. After moving this part behind him, and bringing it in a chronological order, his concentration problem seemed to be resolved.

Time Line Therapy can help to deal with issues from the past or fears for the future and also adjust someone’s personal time line to better fit to his or her needs (Hall & Bodenhammer, 1999). Thus beside doing psychotherapy with concepts (traumas) that date from the past or that are projected into the future (fears) or the present (conflicts), also the personal way in which a client has constructed time can be the focus of this type of psychotherapy.

How a person has constructed time decides many behavioral traits. For instance, motivation varies with how a person envisions the future. Is the future seen as far away and vague? In that case we may expect this person to

experience little or no impulses to invest, study, save, enterprise, close insurances, build a pension or buy stock. The rewards of all these activities are not seen in a compelling manner. However, is the future close, straight in front and clear, this may pull the person to do all the above. Thus the question where a therapist works with is: Where in an individuals' mental space are the images of past or future events located. Since the whereabouts of these have a direct impact on the persons' personality.

Clinical Experiment 8: Modelling the Personal Timeline.

- 1) This experiment takes two people A and B.
- 2) A asks B to imagine three balloons of three different colors. In this imagination, B visualize the word PAST on balloon 1, PRESENT on balloon 2 and FUTURE on balloon 3.
- 3) A helps B to relax and ask his or her unconscious mind to let drift these balloons to the appropriate locations for where the person holds the past, present and future in mental space.
- 4) A helps B to find the time-scale of the past and present, by asking for the locations of important live events: Birth, first day at school, first trip abroad, marriage, etc. Then B does the same for the future: next Christmas, next holiday, pension, etc.
- 5) A helps B to show with his hands where the present is located and how the present connects to the past and the future.
- 6) Now B goes through the same series of steps 2-5 with A.
- 7) When both personal timelines are explored, then start to compare them. What are the most striking differences?
- 8) Next, try out some differing features of the other person's time line. Change it just for a moment by direct suggestion, to explore the effect of the change.
- 9) Return both timelines in their original shapes.

Clinical experiment 9: The now, the past and the future in 3 contexts.

- 1) Pick three of your most relevant life contexts (like: work, family, sport).
- 2) Put a piece of paper on the floor for each context. Step on these papers one by one to connect yourself to a clear experience that stands out for each of these three particular contexts.
- 3) Step into one of these three contexts and explore with the help of your guide, where you sense the past, the present and the future, being in this context. For the sake of precise exploration, it can be helpful to refer to concrete experiences from the past and the future (Christmas last year, Christmas next year, etc.)

- 4) After the past, present and future (time lines) are explored for all three different contexts, take some time to compare the differences. What do the differences tell about how you function in these different contexts? And what tell the similarities about your personality?

7.0 The Self-Space.

Because the awareness of “the self” – besides during some altered states of consciousness – accompanies us wherever we go, it constitutes a steady background to all we do (James, 1890; Gergen, 1998). This continuity leads to a high degree of unconscious familiarity. In other words, we are always sensing ourselves but that is so normal that we only notice it when there is something wrong with it. Like for instance when a person “does not feel to be in his regular self” or “lost himself.” The former may happen when a person uses certain drugs and the latter while he has identified with some role or someone else.

Edward T. Hall, in his “Proximenics” (1966), explored the micro territorial behavior of people: how people sit in busses or in a public library. Other sociologist and social psychologists started to use a variety of expressions for the subjective borders between self and others. The difference in the amount of distance kept during interaction was one of the most fascinating inter cultural subjects. For instance, in Finland people already step backwards at distances where in Brazil people prefer to come three steps closer.

It appears hard to give words to our sense of ‘self’. That is probably so because it is largely located in our less talkative right hemisphere. That is why people tend to resort to vague metaphors. If we stop talking about the self, we are left with the low intensity feelings, images and internal voices that constitute the habituated awareness of our self. These small pieces of awareness are located somewhere in mental space and the patterns found in these bits of experience are the subject matter of the remainder of this chapter.

In spatial cognition research “the self” is often equated with the body (Maselli, 2015). The question where a person experiences his or her self (body) appeared harder than expected: Where is your *self*?

Derks (1997, 2002) found that the kinesthetic part of what people subjectively call their *self*, and where they tend to spontaneously point at, in general consists of an area the size of a tennis ball often located in the chest or the abdomen. The latter resembles more to the qui-point, as this is known in martial arts and oriental psychologies from Yoga to Zen. But this feeling seemed only to be one of the spatial components of the experience of the self.

William James (1890) in his exploration of subjectivity distinguished between the “I”, the “me” and the “self”. And the first therapist that started to look for the locations of these concepts was probably David Grove (Lawley & Tompkins, 2000). He asked his clients: “Where is I?” and “Where is me” and also, “Where is you? – when this was used as synonymous for *me*.

For whatever reason a client comes to therapy, the self (mostly in the shape of too little self confidence) tends to be somehow part of their issue. The having of a problem that one cannot resolve by oneself may result in self worth compromising shame (Andreas, 2001). However, the self can also be the central

issue, like in anorexia, some types of depression or in socio-phobia. Beside that, there is a range of complaints where the self is explicitly mentioned, like for instance: “I lost my self in my relationship.” Derks (2010) distinguishes the following self-related problems:

1. Lack of self-confidence, self-respect or self-worth in a given context.
2. The habitual assumption of inappropriate roles.
3. The regular assumption of conflicting roles.
4. Doubts about one’s origin, culture or mission in life.
5. Problems with the identification with others: who is who?

The self in mental space

Whatever the self related complaints are, a therapist that work on the base of the Social Panorama model will start off exploring how the self of the client is situated in mental space. What makes this rather technical is that clients normally have no clue about a spatial structure of their self. On the one hand this helps to do a good (surprising) “something completely different,” but on the other hand there is no reason to expect the client to assist the therapist in the right direction. Confusion may take over when the therapist hesitates too much and the client loses his confidence.

In practice, this means that the therapist must be rather convinced of what he or she wants the client to do, or else there may be a wave of uncertainty on both sides. The first step to success is a therapist that assumes that the self is made out of four necessary components. These are:

1. The context: The experience of who we are varies with the contexts and the roles and positions we have to take in society. When all goes well, we have already associated the right self experience to the appropriate context. Only in novel situations we need to make up our mind about “who to be” (Gergen, 1998).
2. The kinesthetic-self: To take “the self” as “the body” has been a typical misconception in Western psychology. In oriental psychologies we find a clear notion about “the center of self” or “the feeling of self” or “the kinesthetic self” as being a small area of sensation, most often located somewhere in the upper body (Derks, 2002).
3. The self-connection: The connection between the kinesthetic part and the visual part of the self-experience seems rather crucial. It represents the activity of “being”. Without a connection a person still can have a self-feeling or a self-image, but this does not create the sensation of actual being what is seen in the image. Without a connection, it may just remain a dissociated image of one-self.

4. The self-image: In ordinary everyday discourse, the expression “self-image” shows up frequently. However, many users, even very professional ones, may not regard it to address to a mental picture. The reason is, that self-images are highly habituated (and probably right hemispheric background knowledge) and stay below the threshold of awareness. (Derks, 2002).

The locations in mental space of the above components are searched for in the following manner and order.

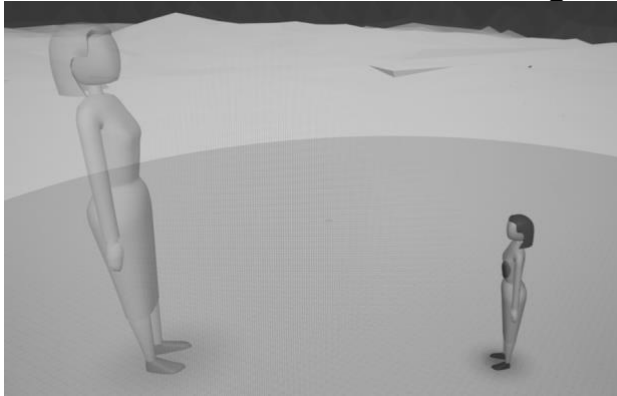
Clinical experiment 10: Exploring the self in 3-D

1. First determine the context in which you want to find the spatial structure of someone’s self experience.
2. Ask the participant to mentally step into this context.
3. Get the participant to ask him/herself ‘Who am I in this situation’ (without him or her needing to give an answer).
4. Ask the participant: “where do you sense your feeling of self”. Then let the person indicate his feeling of self by pointing it out with his fingers. Accept every answer.
5. Let the participant concentrate for a moment on the feeling of self that he found. Then suggest a connecting line forwards in space that originates from the feeling of self. “Imagine some energy (connection) line running forwards out of your feeling of self. Look along this line.”
6. Get the participant to ask him/herself again, ‘Who am I in this context?’ (He or she doesn’t need to answer the question). Then ask the participant to point out, where he/she notices (senses, is aware of, sees) the idea (image) that is (provides, tells, shows) the answer. This probably will be a subconscious image that the person however will be certain enough about to continue.
7. Determine the distance, eye level and the direction of the gaze of this self-image.

Clinical experiment 11 part a: Variations in Self-Image positions

- 1) Ask the group members to move their self-image to 30 meters distance and make it about 30 centimeters small. Call this ‘far away and small’.
- 2) Next ask the group members to move their self-image as close as at 50 centimeters in front of them, then make it 10% larger than life-size and set it in bright sunlight. Call this ‘close, big and light’.
- 3) Ask them to bring this close, big and light self-image right into their body – nose front. Call this ‘turned into oneself’.

- 4) Ask them to send the self-image to a distance of 5 meters and make it 2 ½ times life-size. Call this ‘the sovereign self’ (see the image below).



A sovereign self

- 5) Ask them to imagine five to seven friends to be their personal support group. And then to put these in their imagination behind them, and add to that the experience of these supporters touching them and also speaking to them in a very encouraging way. Call this ‘with support’.

This experiment functions as a preparation for a more experiential exercise, called the narrow alley. For this the group is split up in couples.

Clinical experiment 11 part b: The Narrow Alley

- 1) Two participants stand at 6 meters opposite of each other on either side of two chairs, between which there is a gap of 30 centimeters.
- 2) Now each of the participants chooses, without telling the other, one of their variations in self-image positions from the previous crucial experiment 11a. Then they make it their actual self-image and let each other know when they are ready doing that by a nod.
- 3) Without speaking, they walk towards each other and pass each other, through the gap between the chairs (Johnstone, 1990; Ötsch, 2002).
- 4) They do this three or four times with different self-images. Afterwards they discuss the experience.

Improving the cooperation within oneself.

Many psychologists have divided personality in parts, like Freud's id, ego, super ego or Jung's anima, animus and shadow. But also more recently popular schools like Berne (1961) in Transactional Analysis or Assagioli (1965) in his Psychosynthesis stipulated partitions of the mind. And “Martian anthropology” would certainly include all shamanistic traditions that ever existed as soul cutters, just as their modern versions like Spirit Release Therapy (Baldwin, 1995) in which wondering spirits that are hindering the client

are brought to a location of rest. In all these models one distinguishes spatially distinct components of personality in order to create a new harmony between them.

These days there are many psychotherapists that treat the human psyche as a cluster of relatively independent units that can work in cooperation or can struggle with each other (Stone & Stone, 1993). Parts can cause problems because they were forgotten, repressed, expelled, rebelling, are infestations from an alien origin or can be stranded in early stages of development.

The following experiment is the most complex of all clinical experiments in this introduction. It gives substance to several hypothesis about the make up of the self (identity, personality).

The experiment presupposes:

1) That a person has varying selves (parts, roles, alters) that are classically conditioned to certain contexts. Thus, if the person enters context A, the appropriate feeling of self and self-image for context A will automatically come to the fore.

2) The technique is based on the presupposition that these selves are spatially separated. In the clinical experiment this is operationalized by laying sheets of paper on the floor, to signal the spatial location of a particular self-part. However, in the psychological reality these selves may surround the person more as a kind of 3-D swarm.

3) Not only are the selves swarming around the person, they also automatically reorganize themselves when another context is entered. Like how a team of soccer players is circling around the player with the ball. This makes it possible to reorganize the whole configuration of all a participants' selves in one go within only one particular context: and it is assumed that when the participant enters an other contexts the changes that were made will automatically be taken into account by this system.

4) The technique in the experiment below makes use of the locations of the kinesthetic parts of the selves, but also of the self-images that are connected to these kinesthetic centers. From the previous experiment it will be clear that the locations of the self-images in relation to the feeling of self are for a large part responsible for the power of a self. The power can thus be partly regulated by shifting the self-images.

5) The distance to a particular kinesthetic self and whether it is situated in the center of attention or at the side of that, makes up for the accessibility thereof. By this spatial dimension one can regulate the priority that is given to a

particular self: the closer and the more central the more accessible.

6) Maybe the hardest to belief hypothesis is that, as soon as changes are made in the spatial configuration of self-parts, these changes tend to automatically last. The client does not have to do any rehearsal or additional activities. If the result of the procedure does not immediately hit on resistance, the changes seem to be automatically implemented in the self-system. We already mentioned why in between other things spatial psychotherapy may be so effective: one reason is that the client has no conscious access to this information since it is running in the background in the right hemisphere. By making the client aware of it, he or she can take control where before there existed no such possibility.

Clinical Experiment 11: Self-Design

The objective of this exercise is to adjust roles (personality parts, alters, egos) in such a way that the client is able to better reach his goals in regards to a significant change in career, marital status, financial situation, occupational choice etc.

What change do you want to make in your life?

1. Start with the current context (Who are you here and now?)
2. Get the person to stand on a piece of paper (present self position) and associate in his current self.
3. Check the kinesthetic self, the self-image and the connection between the two.

The exploration of roles. (selfs, ego states, identities)

4. With the client on the current self-position, ask him 'Is this all you are? You are sometimes someone else, who else are you?'
5. The person names another identity, puts down a piece of paper wherever it feels right, writes the name of the role on it, stands on it and associates in it.
6. Check the kinesthetic self, the self-image and the connection between the two in this position.

The further exploration of roles.

7. Search for other roles, starting each time from the current self-position. Have the client put down a piece of paper for each, and check the kinesthetic self, the self-image and the connection in each position.

8. Continue until the person is sure that all the important roles are on the floor.

Design.

9. Take the client to a position outside the area used for putting the selves – further out than the papers on the floor. The latter is called the “Meta Position”. From here the question is asked to the client: “What should be different to help you to achieve your significant change? Which parts should be more or less accessible or which self-image should be bigger, closer or more central?”
10. Now change the locations of the roles round the current self and vary the self-images until the ideal situation for the achievement of the client’s desired change has been reached.
11. Check by going from the current self position into the future. Look for objections and adjust the design accordingly.

8.0 Social Space

Humans are social creatures, so our lives are full of people. To successfully navigate among them we need a representation of humanity that

tells who the others are and how we fit among them. We also need to recall the unique features of individuals in order to recognize them, and we need a way to represent our relationships. The question that is answered by the clinical application of the Social Panorama model is: How do humans keep track of their interpersonal links?

Twenty years of clinical work has led to one conclusion: We unconsciously represent our relationships in a three-dimensional map.

First of all we need to assert that there exist real flesh-and-blood people – with all their specific details, unique characteristics and changeability. They exist as varied and dynamic physical objects in the space around us and we need to compress their enormous complexity to fit our mental processing capacity by generalizing and abstracting. The factual nuance that people possess is beyond what we can store, but, more importantly, taking all of that in to account would disable us to quickly know our position in relation to them. In most interactions we need to instantly know where we stand and what to expect. The above is certainly the reason why people seem to reduce others to mental diagrams that are given a steady location in mental space (Derks, Ötsch & Walker, 2014). In other words, every relevant person is represented as a simplified image that is located in a direction, at a distance and with a certain size. In expressionistic paintings we see an indication of what sort of icons people use to mental spatial index (MSI) the others. The result is a landscape full of representations of people – a social panorama.

The distinction between real flesh-and-blood people and their mental representations, called ‘personifications’, is essential for working with the social panorama model.

The clinical application of the *social panorama* is an extraordinary confrontation with illusive, automatic, high-speed, unconscious thought process. And it shows how people juggle with inter-personal relationships without any conscious awareness. This is why we need to speak of ‘unconscious social cognition.’ Thanks to this unconscious nature, people do not realize that they carry images of other people with them; images that they made up themselves and continue to do so and that they are constantly regrouping and changing. People only become aware of this after they are introduced to the *social panorama* model. Then, relationships become regarded as the products of unconscious spatial representation. Once people know about it, the social panorama seems natural to them since they have been busy using it from early childhood.

This model shows how we assign ourselves a place among others. A place that however often fails to provide us with the best chances in life; and may even severely limit us. Once we grasp this, we may start concur a new area

of choice. This insight will improve our social life, and it will also enable us to help others do the same.

Because almost all human problems have something to do with relationships (have social components), the social panorama model is almost universally applicable. Its systematic nature can clarify the most complicated relational issues. It is an instrument that can be used to work with relationships with loved ones, friends, colleagues, children, parents, strangers, groups, teams, the deceased, ghosts and gods. It is also applicable where self-worth and self-confidence are at stake and it is useful when the relationships between groups, tribes, peoples, political parties, departments and organizations are troublesome.

The concept of mental space plays a central part in the social panorama model. This means that we project representations of other people around and in our bodies. Where we locate (MSI) them within our mental space determines the emotional quality and intensity of the relationship. The influence of those images may provide the experience of support, confrontation, power, helplessness, love, aversion, adoration, possession, togetherness, loneliness, closeness, intimacy, status, servility, dominance, antipathy, ambivalence and hate. All of these seem to arise from giving social images a particular distance, direction and size.

Clinical Experiment 12: Love for a Stranger.

- 1) Think of the person you love most. Start to feel the love for this person.
- 2) Point with your right hand at where you sense the idea of this loved one.
- 3) Find the exact direction, distance, level of the eyes and the direction of the gaze.
- 4) Now enjoy this for a moment.
- 5) Next think of a person that you know, but who is a stranger to you.
- 6) Feel the feeling that comes up by thinking of this person. Then find the location of the image of this person.
- 7) Now move this image of this stranger to the exact location of your loved one and let also the eyes be on a similar level and the direction of the gaze in the same direction.
- 8) When you have felt what the effect of this move is, please bring everything back into its original shape.

Clinical Experiment 13: General Social Panorama

In this experiment a person's relationship with humanity as a whole is what is explored. Even though this relation is abstract and functions on an unconscious level, it can be revealed and adjusted by the spatial process below.

1. Ask the participant to think about all the people in the world (humanity at large), in a natural, normal way. Let the participant gesture to where humanity is experienced.
2. Then find out whether the participant has an associated or dissociated image of humanity. To establish this one asks: Do you experience humanity separate from yourself, or do you feel yourself to be included or surrounded by humanity?
3. If the participant has a dissociated (not self including) image then ask him or her, to change it into an associated one. This is done, by asking him or her to find himself or herself in his image of humanity and then to step into them selves.
4. Then ask what the average distance to the mass of people is. And in what direction this mass is sensed? What is the area filled by humanity? Is this area located it in front, around, at the side, behind or somewhere else?
5. Then let the participant explore the average level of the eyes of humanity in comparison to their own eyes. Does the participant need to look up, at equal level or downwards? And how much?
6. After the participant has become aware of the spatial dimensions of their spatial model of humanity, this leaves them with two logical dimensions to explore a bit further. Thus this experiment can be extended by, first increasing and then reducing the distance to mankind. Also the general level of the eyes can be made higher or lower.

9.0 Linguistic Space

Generations of psychologists (and philosophers) saw language as the foundation of reason, and reason as the basis of cognition (Droste, 1977). However, in mental space psychology the government of *king-language* is thrown over by the mighty *emperor 3-D*. But was space not already ruling way before language came on stage?

Spatial cognition steers spatial behavior. Humans, and all other moving creatures, do possess such spatial behavior, and it also let them send and receive tell tale signals. Movement in space can communicate an endless variation of meanings. For instance, when my dog goes tail wagging to the door, this means to me: “Take me out please?” When I walk in the direction of the fridge it means to him: “A snack!”.

When we assume that all mental activity is based on a spatial organization, then one may wonder if this also applies to language. This section tries to answer this question with the help of linguistic sources and most of all clinical input.

Words, outside and within the context of psychotherapy, are 1-D series of sounds or symbols. They get their meaning from the 3-D experience they are associated to by means of classical conditioning. Thus theoretically, when people speak, they trans-code 3-D experience into 1-D sentences and when they listen, they do the opposite. However, it appears that the transformation of thought into language and back runs in close synchrony, extremely fast and largely outside of awareness. That all together made it a challenge for science.

During neuroscientists were figuring out the brain areas of language processing; cognitive linguists studied how language is used to express 3-D information. For instance, much of what was asserted in the previous section about social space is supported by ordinary relational expressions. For instance: “I look *up* to her”, “We drifted *apart*” and “They are *backing* me up.” These are spatial utterances that help to convey clear messages about the state of affairs in ones social life. Several researchers stressed how *spoken words* in combination with *gestures* may convey even more about the 3-D character of what is talked about (Lakoff, 1987; Tversky, 2010). And there is evidence that suggests that gestures alone can be just as meaningful as gestures in combination with words.

Signing (sign language) probably exists from the time *homo-erectus* had its paws free to gesture. Some call it the *universal language*, pointing at how deaf people from different continents can understand each other after a couple of days, only limited by their cultural barriers. *Natural sign language* contains many gestures that are just spatial simulations of what is communicated about (Keatz, 2015; Sachs, 1988). These signs are not so much *symbolic* but rather the re-enactment in non-verbal scenes and mimed representations of the topic at hand, all down scaled to fit the little *theatre* in front of the torso and the face.

Some believe that when there existed no spoken languages on earth, *natural sign language* would still connect humanity as it did before the Babylonian confusion of tongues.

The *spatialization of form hypothesis* as put forwards by George Lakoff in 1987, brought the whole issue of grammar in a shape that fits to mental space psychology. Lakoff states that grammar (re-) creates the *spatial structure* among concepts. He saw grammar as that what links mental images into meaningful 3-D constellations. In the development of cognitive linguistics both contributions, Fauconniers' *mental spaces* and Lakoff's *spatiality of form* point at a similar psycho-linguistic reality. In this view grammar acts like compression in video (a codec). The original conceptual data in the brain are in 3-D. Grammar helps to compress that into the 2-D of graphics or in the 1-D of language. The same grammar does the opposite in the listener. From the 1-D linguistic structure the listener recreates 3-D imagination with the help of grammar.

A sentence like: "The duck eats a clam" receives meaning from the reader composing this scene in mental space. The grammar found in *natural sign language* can help to bring the images of *duck* and *clam* (created with the hand gestures) in their rightful spatial positions. The positioning of the concepts in sign language is more immediate, compulsory and analog, than how a spoken language achieves this end. For instance, one can sign in one go how a smaller, lower rat jumps on top of a much bigger cat and bites it in its neck – a spatial scene that leaves no room for misunderstandings. But in the realm of spoken language the listener might just as well visualize the rat as being hunted by a kitten. The rat then un-intentionally uses the rattrap that was set for him by a human. In the rush to his barrow the rat runs safely around the trap. But the kitten accidentally steps on the trap, which immediately snaps tight around its paw and cripples him: the rat catches a cat.

Space, language and psychotherapy

Language was the starting point in the development of Neuro-Linguistic Programming (Bandler & Grinder, 1976, 1979). By analyzing frequently used language patterns in the sessions of several reputed psychotherapists – like Perls, Satir and Erickson – the so-called Meta Model was created, that consists of a list of question-formats. The diverse questions in the Meta Model are mere reactions to the grammar in the clients' utterances than to their conceptual content.

It was only after Bandler (1987) introduced "sub-modalities" that the question "where" became a regular appearance in NLP. This "where?" is frequently used to locate emotional feelings in the body or to find the places where voices speak from, but foremost to locate mental pictures. However, also the positions of words can be explored: "Where is *honesty*?" or "Where is

freedom?” or “Where do you see *safety* in your mental space?” Then the client may gesture at the specific location where these words are sensed.

The mental processes in a relaxed hypnotic trance appear to be slowed down and reduced, which intensifies the impact of the hypnotherapist’s words. Although the intensity and sensory quality of hypnotically induced imagery exceeds that in a regular state of consciousness, afterwards the person may not be capable of telling what exactly happened. Thus, the meaning of the hypnotherapists’ words may be intensified, but they cannot reflect much on what is going on inside nor can they report about that later. This inability seems mainly due to hypnotic amnesia. This has always been the central problem in clinical experiments with hypnotherapy.

To use hypnotic trance in psychological research – for which there could be many good reasons – the experimenter is limited to paradigms with a small bandwidth. For instance, the participants are not doing well behind a computer screen or a paper questionnaire during trance. What works best is that the experimenter him or her self asks the questions as part of the hypnotic process. Although this seems extremely unreliable in the eyes of lab-researchers, it has provided the field of hypnosis with a very solid body of knowledge over the last century (Kroger, 2008; Spiegel & Spiegel, 1978; Hillgard, 1977). The consistence in the phenomena that came to the surface as a result of clinical experimentation with hypnosis, screams and yell to be integrated into the regular psychological curricula as they are used at universities around the world.

Clinical Experiment 14: Critical Hypnosis.

- 1) First the subject is brought into a deep hypnotic trance. Preferably use a fragmented induction for this purpose (Bandler & Grinder, 1982).
- 2) When the participant shows the signs of deep hypnosis, the following suggestion is given. “I will tell you about my holiday. Please pay attention to where the images show up in the space before you. When I ask you to do so, you point with your right hand at where an image appears to you.
- 3) Tell the participant a story that is rich in sensory qualities. Use 5 words that must raise clear images. (like: the tree, the table, the cook, the car...)
- 4) Ask the participant 5 times to point out the location of a used concept for you. “Point with your right hand at where you notice the tree?”
- 5) Awake the participant to report on what they remember about their experience and if they still know where these concepts showed up in mental space.

10.0 Spatial metaphors

Metaphor is a cognitive phenomenon, in which the meaning of one domain is represented in another domain. Metaphors may stay contained within an individual's private thought processes, but they are also frequently verbalized with a variety of communicative goals (Derks & Hollander, 1996). For instance, a person is talking about *plants* (= domain of vegetation) but wants to explain a point to his colleague about the *development of people* (domain of education): "Plants need dirty water to grow well," is what this person says, but this is meant as an equivalent of: "People will develop themselves best when stimulated with enough challenging and chaotic tasks". In metaphoric communication one finds a large amount of spatial expressions – that is the reason for this chapter on *spatial metaphors*.

Spatial metaphors use the domain of 3-D concepts and constellations to represent things that by them selves do not need to have a 3-D spatial nature. For instance, one may call a book "traveling new pathways" or "digging into deeper layers"; but real books cannot travel or dig. A bundle of paper on its own, travels nowhere and cannot shovel its way into the "depth". Of course, caves can be deep just as lakes, crevasses or hidden treasure. But most people still understand what it means when a book *travels untrodden tracks* or *digs deep*, and most may not even notice the spatial metaphor.

Mental space psychology builds on the view, that *the model of the world* that, people create in the imaginary sphere around them is their primary simulation of reality (Derks, Ötsch & Walker, 2014). In other words, the mind cannot reach a more similar (better, more accurate, verisimilar or more fundamental) representation of the world, than it does in its seen, heard, smelled, tasted and felt 3-D map of its 3-D environment (Spivey, Richardson and Zednik, 2010). In NLP the concept of "The Model of the World" is used according to Alfred Korzybski (1934), without any reference to its dimensional shape. Mental space psychology however, claims that this model is primarily 3-D. And it seems probable that the mind creates this complex spatial model for the larger part within its right hemisphere. There it functions in the background as a kind of silent library of knowledge about the universe (McGilchrist, 2009).

Superimposed on this as good as possible 3-D reproduction of physical reality, can be all kinds of derivatives, abstractions, generalizations and transformations. Among these are bold generalizations in image, symbol and word that radically simplify the real world experience. These schemas, caricatures and formulations however do have the power to overrule the faint and vague but often more realistic layers of 3-D representations. They may owe their strong impact to their simplicity and a superior signal to noise ratio: they stand out with clarity and high contrast against the more nebulous background of other but more accurate types of knowledge. It is probably due to the difference in neural conduction between the hemispheres, and the other hemispheric structures that, the left frontal and left temporal regions of the

cortex can dominate the rest with their more 1-D linguistic and 2-D- schematic focused knowledge (McGilchrist, 2009).

When we assume space to be the central organizing principle in cognition, it is understandable that when people try to express themselves, they may also do this in the form of spatial metaphors – especially when they cannot express themselves in a literal sense because of the complexity of the subject matter.

Spatial metaphors can help create the strong simplified superimposed imagery that overrules other types of mental representation. However, spatial metaphors may also find a place in between other 3-D concepts that are functioning more in the background.

The above means for cognitive linguists (Lakoff & Johnson, 1999; Bergen, 2012) and for social scientists that follow the ideas of Alfred Korzybski (1933) like Bandler & Grinder (1976), that, on top of the 3-D embodied simulation of the world there can exist more indirectly acquired cognitive maps with a lesser resemblance to the real world. These concepts can range from linguistic formulations and 2-D schemes to virtual 3-D worlds of delusional imagery with multisensory qualities that fail to match to any one fact in universe. In many of the world's regions such fantastic constructions – like spiritual pantheons – are transferred by culture at young age and may even rule people's lives (Derks, 2006). Some spatial metaphors structure the world in a very powerful way: for instance in *heaven* and *earth* or offer hierarchical schemas of society. Also the division between groups, and the belonging to (in-) groups or the being of an outsider, falls in this category of spatial metaphors.

Scientists may use spatial metaphors for *science* and *no science*, as in *within and outside of science*. Their own specific field of research will be probably represented in spatially organized schemas with arrows and boxes. And one can argue that science itself is the activity of reducing intuitive 3-D knowledge about the cosmos, into 2-D metaphors. This process of (dimension) reduction is driven by the desire for the action-power that 2-D knowledge can provide. Symbolic simplification enables reasoning, logic and calculus and may show a person immediately what to do, what to believe and who to follow. The transformation of the more complex and faint 3-D imagery into simpler and clearer 2-D formulations helps to create technology and an organized society, but often this goes at the cost of the connection to the intuitive 3-D knowledge (McGilchrist, 2009).

Language has received a tremendous interest from philosophers, linguists and psychologists, largely because modern man has regarded his possession of language as one of the unique features of being human. Language has shown to be a tool to create, conceive, generalize, structure, communicate, manipulate, control, lie and deceive. But at the same time language constitutes something

that can also estrange humans from the sensory facts as they present themselves in the actual here and now. Before the spatial psychotherapies arrived on the scene, language was the primary tool in psychotherapy.

The Meta Model (Bandler & Grinder, 1976) is a therapeutic tool that was largely based on the above philosophy (Korzybski, 1933). The meta model can be used in psychotherapy to breach too over generalized, distorted and impoverished mental constructs and force the client to face more realistic (3-D) representations of what goes on in life.

When we assume that language develops on top of embodied (sensory) knowledge, this does not mean that this is a one-way street. Language can also help create new, never felt, never heard of, never seen experiences. People can also use language to describe (and create) new spatial ideas that are metaphorical for complex issues (Narayanan, 1997). Who looks and listens at natural conversations, finds that these are riddled with spatial expressions that are not meant to be taken as literal (Lakoff & Johnson, 1980), and these types of spatial expressions occur just as often in psychotherapy. “Our competitors are blocking our access to the market,” can fit in a business meeting. “My fears are standing in the way of finding intimacy,” can be heard in a therapists’ office. Both expressions share the same spatial metaphor: of an obstruction on the way to a desired goal (the market, intimacy). From the previous section, on linguistic space, follows that a person that hears “...standing in the way...” must create a spatial piece of imagination to grasp its meaning.

Metaphors from the therapist

David Gordon (1984) looked for linguistic patterns in successful (healing) metaphors in psychotherapy. He was inspired by the puzzling examples provided by Milton Erickson (Haley, 1976; Bandler & Grinder, 1975; Lankton & Lankton, 1983). Erickson was telling a host of stories and case examples to his clients and students (Rosen, 1982). These anecdotes stood central in his teachings and therapeutic interventions. And he could present a compendium of follow up on cases that proved that his metaphors were a major impulse to the cure.

From the Martian Anthropologists perspective the metaphors told by a therapist are just another kind of “something completely different”. And indeed some metaphor-telling-therapist argue that the exact content and structure of the stories told by a therapist at the time his client is connected to his problem state, is far less critical than one would expect. Some claim that everything goes as long as it carries the client away from his habitual problem behavior. The reported surprisingly strong therapeutic effect of therapeutic metaphors might just as well come from a radical shift in attention, away from the domain of knowledge and skills in which the client was searching in vain for a solution. By

imposing a metaphor on the problem state, such a move will redirect the clients search in an inescapable way. There seems to exist a “story listening state of mind” in most people, that, some hypnotherapists connect to bedtime stories in early childhood. A metaphor may take the client from his habitual dead end mind track and stimulate alternative forms of coping (Austin, 2013).

In a more technical formulation: The metaphors added to the problem-state enlarge the search range by adding a greater variety of possible analog connections (Derks & Hollander, 1996). The greater variety in sensory qualities (sub-modalities) that become part in the searching process may hold the key to the finding of a broader set of solutions (in the shape of new neural connections to come into being). So one may say that the story, when listened to in the context of the problem state, enhances the chance of a lucky shot in the clients’ latent capabilities (resources). Having said that, does not imply that some metaphors stemming from the therapist will not do a better job than others. Thus it is not just a matter of telling nonsensical stories in the light of the clients’ misery. However, in the middle of an intense search for a solution, a client may be very open to any type of information; and may show a great flexibility and creativity to make use of whatever is on offer.

Metaphors from the client

Beside the therapist being the one who is telling therapeutic metaphors, we frequently encounter the opposite: The client expresses him or herself in metaphors when explaining his issue. Several therapist reported good results when they responded in a (matching) metaphoric way but also in literal way to these stories: as if the metaphor itself was the real problem. These successes resulted in the development of methods to facilitate clients to create a metaphor for their issue, when they failed to use spontaneous ones.

A great leap was made when the work of David Grove (1998, 1989) was studied by James Lawley (see photo below with Grove in the corner) and Penny Tompkins (2000). This project led to a method called “Clean Language”, which had a substantial impact on the field of psychotherapy in the United Kingdom and its former colonies.

When the client is connected to the problem state, the therapist that uses *clean language* will ask: “And your problem is like what?” and by that help the client to create an analogy-metaphor for his problem. Next the therapist will help the client to elaborate on and extend this metaphor as much as possible. It appears that the more the metaphor is unfolded, the better the chances are that the client will find creative connections that help to solve the issue. One of the main questions that are used in Clean Language is: “And whereabouts is (the metaphoric element named by the client)”. Clean Language developed into a

method that is called Clean Space, and that was introduced in the section on awareness space.

Andrew Austin (2010) recognized the typical coping styles of his clients in their spontaneous metaphors. The same was true for the metaphors that he stimulated them to compose about their issues.

Austin recognized a series of general spatial patterns that tend to appear in his clients' metaphors that showed how they were moving unsuccessfully towards a solution. This led him to focus more on what he called "metaphors of movement". He developed a method to visualize the ineffective travels. And he found that changing these "metaphors of movement" offered a point of leverage that could radically change his clients ways.

The inappropriate coping style is easily recognized in a metaphor that shows, *from where* and *in what manner* the client tries to move towards a *desired state*. Austin puts forward that, the movement in space in the clients' metaphors, is a set up for a rigorous therapeutic effect. For instance, the client may describe his situation as "swimming towards an island that moves further away the harder he swims." Austin (2013) writes:

The Metaphors of Movement work aims, to explore an entirely different area than that of human misery - it aims to strategically transform the fundamental coping behaviours inherent in the client's behavioural system. The work does this by:

- 1) Connecting the client to behaviours that commonly occur outside of their conscious awareness.
- 2) Exploring the mental landscape ("the map").
- 3) Examining the coping behaviours employed within this mental map.
- 4) Testing the effectiveness of these behaviours in the real world ("the territory").
- 5) Designing and teaching more suitable behaviours to be applied in the client's map and territory.

Derived from the richness of Austins work comes the next clinical experiment. To stimulate the client to draw or help them make a drawing is already a very strong "something completely different". But it also enables a client to express something of his or her 3-D experience in a richer form than that language enables them to do. The drawing lacks the ambiguity of grammar and stimulates right hemispheric creativity.

Clinical experiment 15: The Problem State Metaphor Drawing.

- 1) The participant thinks of a problem that is vague, unclear and thus hard to express in words.
- 2) Now talk about this anyway, for about one minute without solving it.
- 3) Now the experimenter asks: "And all of that is like what?" (Or another question that brings an analogy for the problem to mind).

- 4) It is crucial for the experimenter to accept everything that comes up as an answer.
- 5) Whatever the analogy for the problem state is, the experimenter helps to focus on the spatial aspects: “whereabouts are/is the described metaphoric element(s) in mental space?” Next, these utterances are specified by asking: “Where are you in this story?” “Where do you come from?”, “How do you transport yourself?” and “Where are you going to? (The desired state).”
- 6) The experimenter accepts all answers, to next help the participant to create a drawing that includes all metaphoric elements (but no literal aspects of the problem).
- 7) While making the drawing the focus is on exploring what is blocking the movement towards the desired state.
- 8) Finally the experimenter asks the participant to change the image in such a way that the desired state (the goal) can be reached. The latter may take the better part of the time.

11.0 The Spiritual Panorama

The social panorama is not made up exclusively of living people. Personifications can also be dead people, spirits, gods, ghosts and virtual idols. Also, the personification of things often goes even further than dolls or idols – cars, boats, cameras and computers are often treated as social units (particularly when they go wrong). Social abstractions, like socialism, the state, the economy or the market can also be visualized as human-like. When we speak of someone's spiritual panorama, then we speak of the landscape of all personifications that do not represent living people.

Spiritual Experiences

In prehistoric times people recognised the healing, integrative and conflict-solving properties of very intense experiences on the highest level of abstraction – what we call spiritual experiences. These experiences have superlative sub-modalities.

In the social panorama we distinguish between:

- 1) Personified and non-personified spiritual experiences. (Entities or not?)
- 2) Collective and individual spiritual experiences (Alone or with a group?)

Grief

When someone we know dies we cannot, because of the limits of our brain in that respect, expunge them entirely from our social panorama. The memory remains. However, it is essential for survivors to be able to make the distinction between the living and the dead. To do this when someone dies, it is necessary to change the sub-modalities of his personification in such a way that he is immediately recognizable as being dead. This is usually done by making the personification fainter, raising it, making it lose contact with the earth or making it transparent.

If the dead person was a part of someone's intimate circle, particularly in the most important place, then the transformation of the personification into an appropriate 'dead-loved-one' location is not always easy. Often bi-locations occur, in which one part of the personification is still in the intimate circle and the other, for instance, has already been put in heaven (far away and high up).

From the above we can see grief therapy as putting the personification of the dead person in an appropriate location. Given the great influence of culture and religion on what is seen as an appropriate location, we will need to look first at what that is for the client.

What works well is:

Find the location of a peaceful dead person. (e.g. grandfather)

Move the personification of the problematic dead person to more or less the same place and then work with all objections that may come up.

Gods and Spirits

Social cognition is a part of social psychology, while for many people entities, gods and spirits have to do with 'supernatural' matters. In the social panorama model we see them as part of the subjective social experience and this often meets with resistance. People are often opposed to the 'psychologising' of spirituality. This is understandable; in the first place it removes the supernatural character, which takes away their hope of life after death. Secondly there are people who set themselves up as unique merchants of spirituality; they see and present themselves as a sort of 'chosen ones' because they have a greater than average connection with higher things, with gods or spirits. Often they make a living out of their unique relationship with things spiritual. 'Psychologising' (or 'anthropologising') removes their authority and the legitimacy of their income. People who reject a psychological explanation for spiritual phenomena often develop very refined, creative, metaphorical reasoning that shows the psychologist to be either immoral or backward.

Possession

The concept of possession occurs in almost all cultures. It is usually seen as a spirit that takes control of a person. In this view the person loses control over himself. The parallel with parts work in NLP has been discussed before; parts and personifications are actually the same thing and it is just a question of the difference between self and other.

In the social panorama model we recognise the following distinctions in possession:

- Voluntary versus involuntary possession.
- Short-term versus long-term possession.
- Controlled possession versus uncontrolled.
- Unlimited versus context-bound possession.
- Empowering versus pathological possession.

We can understand possession as a phenomenon of identification in which the personification of the spirit is situated within the body and /or in the place of the kinaesthetic self. Because the spirit is represented more prominently than the self-image, the person, according to the law of the dominant personification, goes into second position with the spirit. Essential is:

The relative size and nearness of the spirit personification and the self-image.
The accessibility, during the possession, of a larger self. This can be called up by the person himself or with the help of someone else. A large self-image makes it possible for the person to get back into himself and make the spirit leave.

Shamans who practise in-context controlled possession often have their self-image inside themselves (turned in on themselves).

12.0 Dealing with Traumas

When working with the family panorama, clients often come across old, unresolved negative experiences. This can prevent any progress with the family panorama so it is to be recommended that the traumas be solved first. To do this we work on the following assumptions:

A trauma is an experience in the past for which the client had no resources, either before, or during or after the experience. Solving a trauma consists of finding the missing resources.

Emotion (crying) is an indication of having found the point of missing resources. Expressing the emotion without finding the resources has absolutely no therapeutic worth. The emotion is the signal that the client has arrived, in his thought process, at the point where the resources are needed.

In general there are three sorts of resource that the client can need in order to deal with a trauma.

- a. Resources before the trauma that would have made it manageable. (resources in the prologue)
- b. Resources that would have been of use during the situation.
- c. Resources that would have been useful after the event (resources in the epilogue)

As the client becomes more emotional it is better to let him search for the missing resources exclusively from the sideline, dissociated. It is essential to keep the client from regression, which is often difficult. You need firm anchoring of the dissociated observer situation.

The reason for not allowing regression is that resources are easier to find for a client who is experiencing himself as an adult.

Have the client decide, from the sideline, what resources were missing before, during and after the traumatic situation. Let him find concrete examples in his own (recent) life of these resources. If that is difficult, then use model examples. Have the client transfer these evoked resources to his younger self – without associating in that younger self.

Only when the client, from the sideline and dissociated, has a good image of how the previously traumatic situation could go better with the added resources (positive emotions) do we allow the client to associate in his younger self.

We can take it as a sign that the trauma has been successfully dealt with when the client, associated in his younger self, is able to tolerate the situation. By traumas that could occur again it is vital to do an extensive future pace.

Dealing with Trauma (associated)

If it is difficult to get a client to dissociate, an associated method can also work well.

A recent example of this is the way Tom Stone uses this technique with traumatised war veterans. Usually the exclusion (repression) of the negative feelings is a part of the problem; the client is paralysed.

Call up the traumatic situation so that the client experiences the negative reaction.

Test whether the client actually feels the reaction in his body.

Ask him to close his eyes.

Suggest to the client that he can feel an energy field in his body around the negative feeling from the trauma. Ask the client to describe where this energy field is located.

Let him find where the energy field is strongest and where it is most concentrated.

Ask him to notice, very exactly, where the strongest negative feelings are.

Suggest that the surrounding energy can stay the same for a while, but the negative feelings will begin to fade.

As soon as the negative feelings fade, have the client concentrate on the place where the energy field is most intense until there is neither energy nor negative feeling left.

Comment:

This approach belongs to the category of techniques by which the therapist calls up the problem feeling in the client and then gets him to do something totally different from what he is used to. The possibilities for variation are endless. We have to assume that during the 'other' behaviour the chance of finding resources (unconsciously) increases strongly.

